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LegalClarity**FAMILY LAW**

At What Age Can a Child Make Their Own Decisions?

Children gain different rights at different ages — here's what the law actually says about when kids can make their own choices.

LegalClarity Team · Published Apr 1, 2026

Children gain legal decision-making power gradually, not all at once on a single birthday. Different rights unlock at different ages — from opening an online account at 13, to working a first job at 14, to signing a binding contract at 18. The exact age for each milestone depends on the type of decision and, in many cases, the state where the child lives.

The Legal Age of Majority

The "age of majority" is the point at which someone is legally an adult. In most of the country, that age is 18. Alabama and Nebraska set it at 19, and Mississippi sets it at 21. Once you reach the age of majority in your state, your parents no longer have legal authority over your decisions, and their obligation to support you generally ends.

Turning 18 opens the door to a long list of rights and responsibilities. You can vote under the 26th Amendment, sign contracts that bind you permanently, serve on a federal jury, and make your own choices about where to live, where to work, and whether to continue your education.¹ You also become fully responsible for your own debts, legal disputes, and financial obligations.

Child support is one area where parental financial responsibility can extend past the age of majority. If a child is still finishing high school, has a significant disability, or the parents previously agreed to extended support, courts in many states will keep the support order in place beyond the child's 18th birthday.

Medical Decisions

Parents generally must consent to their child's medical treatment, but the law carves out several important exceptions where minors can make their own healthcare choices.

The Mature Minor Doctrine

A number of states recognize what's known as the mature minor doctrine, which allows a minor who demonstrates genuine understanding of a proposed treatment to consent without involving a parent. Courts and providers look at the minor's age, intelligence, and the seriousness of the treatment when deciding whether the minor is mature enough. The specific age thresholds and requirements vary widely — some states set a floor of 14 or 15, while others leave the determination entirely to the treating physician's judgment.

Consent for Sensitive Health Services

Even in states that don't formally recognize the mature minor doctrine, statutes commonly let minors consent to treatment for sexually transmitted infections, pregnancy-related care, substance abuse, and outpatient mental health services — often starting as young as 12. The rationale is straightforward: teenagers who fear their parents finding out may avoid treatment entirely, and untreated STIs or addiction create public health risks that outweigh the usual parental consent requirement.

When a minor lawfully consents to their own care, federal privacy rules shift as well. Under HIPAA, a parent is generally not treated as the minor's personal representative for that specific treatment, meaning the provider is not required to share records with the parent.

Psychotherapy notes — the detailed notes a therapist keeps from private counseling sessions — are even more tightly protected. Parents generally cannot access those regardless of the circumstances.²

Emergency Treatment

In a genuine emergency, consent rules take a back seat. Federal law requires any hospital with an emergency department to screen and stabilize anyone who shows up, regardless of age or whether a parent is available to sign forms.³ Hospital staff should not delay a screening exam or stabilizing care while trying to reach a parent. The legal principle behind this is implied consent — the assumption that any reasonable parent present would authorize lifesaving treatment for their child.

Education Decisions

Every state requires children to attend school, but the age at which compulsory attendance ends ranges from 16 to 18 depending on where you live. In states where the cutoff is 16 or 17, a teenager can potentially leave school before graduating, though most states attach significant conditions. Florida, for example, requires a 16-year-old to file a formal declaration acknowledging that dropping out will likely reduce their earning potential, signed by both the student and a parent. Other states require a meeting with school officials, written parental consent, or both before a minor can withdraw.

The practical upshot: even in states where a 16- or 17-year-old can technically stop attending school, it's rarely as simple as just not showing up. And in states where compulsory attendance runs through age 18, the decision effectively gets made for you — you're legally required to be in school or an approved alternative program until adulthood.

Employment and Labor Rights

Federal law sets 14 as the minimum age for most non-agricultural employment, with strict limits on the hours and types of work younger teenagers can perform.⁴ Fourteen- and 15-year-olds may only work outside school hours, in non-manufacturing and non-hazardous jobs, for limited periods each day and week.

At 16, the restrictions loosen considerably. Sixteen- and 17-year-olds can work unlimited hours in any occupation that hasn't been declared hazardous by the Department of Labor.⁵ The hazardous occupation list is extensive and includes operating power-driven machinery, working in mining or logging, handling explosives, and jobs involving exposure to radioactive materials.

⁶ Those restrictions disappear entirely once you turn 18.⁴

One notable exception: children of any age can work for a business entirely owned by their parents, though even then, no one under 18 can work in a hazardous occupation.⁴ Many states layer additional protections on top of these federal rules, so the actual limits for a particular teenager may be tighter than what federal law requires.

Driving

Learning to drive is one of the first major steps toward independence for most teenagers, and every state uses a graduated licensing system that phases in driving privileges over time. The minimum age for a learner's permit is typically 15 or 16, though a handful of states allow permits as young as 14. During the learner stage, a licensed adult must be in the car.

An intermediate or restricted license — allowing a teenager to drive alone with certain limits like nighttime curfews and passenger restrictions — generally becomes available at 16. Full, unrestricted driving privileges come later, usually at 17 or 18, once the teenager has held the intermediate license long enough and logged enough supervised driving hours. This graduated approach reflects the same principle behind most of the ages discussed here: autonomy expands as demonstrated competence grows.

Financial and Contractual Matters

Contracts and the Right to Walk Away

Minors have limited legal capacity when it comes to contracts, and the law deliberately tilts in their favor. A contract signed by someone under the age of majority is "voidable," meaning the minor can cancel it — before or within a reasonable time after turning 18. If a 16-year-old buys a car on a payment plan and later decides to back out, they can generally void the deal and return the car, even if they've been driving it for months.

The major exception involves necessities — things like food, clothing, shelter, and medical care. A minor who receives necessities can be held to the contract, because allowing minors to walk away from those agreements would make vendors unwilling to deal with them at all, which would ultimately hurt the minors themselves.

Bank Accounts and Custodial Assets

Minors can own property and have money set aside for them, but until they reach a specified age, someone else manages it. Custodial accounts under the Uniform Gifts to Minors Act or the Uniform Transfers to Minors Act are the most common arrangement. An adult custodian controls the account until the child reaches the transfer age — typically 18 or 21, depending on the state and the type of account — at which point the now-adult beneficiary takes full ownership and control.

Credit Cards and Independent Borrowing

Opening a credit card in your own name is one financial milestone that doesn't arrive at 18 for most people. Federal regulations require card issuers to evaluate whether applicants under 21 have an independent ability to make minimum payments, and the issuer can only count the applicant's own income — not a parent's household income that the applicant merely has access to. In practice, this means most 18-year-olds without a steady paycheck will need a cosigner or an authorized-user arrangement on a parent's account. At 21, the rules relax and issuers can consider any income you reasonably expect to access.⁷

Taxes and Dependency

A child's age also determines their tax status. Parents can claim a child as a qualifying dependent if the child is under 19 at the end of the tax year, or under 24 if the child is a full-time student.⁸ That dependent status has real consequences — it affects the parent's tax credits and the child's standard deduction.

For children with investment income, the "kiddie tax" applies another layer. If a child's unearned income (interest, dividends, capital gains) exceeds \$2,700, the excess is taxed at the parent's higher rate rather than the child's typically lower one.⁹ This rule applies to children under 19, or under 24 if they're full-time students, and prevents families from shifting investment income to children to take advantage of lower tax brackets.

Online Privacy and Digital Consent

Age 13 is the most consequential threshold in a child's digital life. The Children's Online Privacy Protection Act defines a "child" as anyone under 13 and requires website and app operators to obtain verifiable parental consent before collecting personal information from those users.¹⁰ This is why nearly every social media platform, email service, and online game sets a minimum account age of 13 — it's not an arbitrary choice but a direct response to federal law.¹¹

Once a child turns 13, they can generally create accounts and agree to terms of service on their own. That said, COPPA is a floor, not a ceiling. Some platforms set their own minimum ages higher, and a growing number of states have passed or proposed laws requiring parental consent for social media use by older teenagers as well. The landscape here is shifting quickly.

A Child's Voice in Custody Proceedings

During a custody dispute, a child's preference matters — but it doesn't control the outcome. Courts use the "best interests of the child" standard, which weighs a long list of factors including each parent's stability, the child's established routine, and the quality of the child's relationship with each parent. A child's stated preference is just one piece of that puzzle.

How much weight a judge gives to the preference depends heavily on the child's age and maturity. A 15-year-old who can clearly articulate why they want to live with one parent will generally carry more influence than a 7-year-old who wants to live with the parent who lets them stay up late. Judges also look at whether a child's preference appears to be their own or reflects coaching by a parent — a dynamic that arises more often than anyone involved would like to admit.

In contested cases, courts often appoint a guardian ad litem — someone whose job is to independently investigate the situation and recommend what's best for the child. This person isn't the child's lawyer and doesn't simply echo the child's wishes. Their role is to act as a factfinder for the court, sometimes recommending something the child doesn't want if the evidence points that way. A child never has the legal right to choose which parent to live with until they reach the age of majority, but their voice gets louder and harder to overrule as they get older.

Marriage

Marriage before 18 is still legal in most states, though the trend is firmly toward restricting or eliminating it. The most common minimum age with parental consent is 16, and some states also require a judge's approval. A growing number of states — roughly 16 plus the District of Columbia — have banned child marriage entirely by setting the minimum at 18 with no exceptions. Since 2016, more than 30 states have strengthened their marriage-age laws in some form.

Where child marriage is still permitted, the minor who marries often gains many of the legal rights of an adult, similar to emancipation. But the decision to marry is one that parents and courts heavily gate-keep — a 16-year-old can't simply walk into a courthouse and get married without consent from a parent and, increasingly, approval from a judge.

Civic Milestones at 18 and Beyond

Voting, Jury Duty, and Military Service

The 26th Amendment sets the minimum voting age at 18 nationwide, with no exceptions and no parental consent involved. Jury duty follows the same threshold — you must be at least 18 and a U.S. citizen to serve on a federal jury.¹

Military enlistment allows a one-year head start. You can join the armed forces at 17 with parental consent, or at 18 without it.¹² At 18, men are also required to register with the Selective Service System, a legal obligation that runs through age 25.¹³ Failing to register can affect eligibility for federal student aid, government employment, and citizenship applications.

Passports and Firearms

Teenagers aged 16 and 17 can apply for an adult passport, but the process still requires showing that at least one parent or guardian is aware of the application — through a co-signature, a signed note, or paying the fee.¹⁴

Firearm purchases from licensed dealers follow a split age rule under federal law. You can buy a rifle or shotgun at 18 but must wait until 21 to purchase a handgun.¹⁵ Some states impose higher age requirements or additional restrictions beyond the federal baseline.

Emancipation

Emancipation is the legal process that lets a minor become an adult before reaching the age of majority. A court order formally severs the legal relationship between the minor and their parents, ending both the parents' authority and their obligation to provide financial support. In return, the emancipated minor takes on full responsibility for their own housing, finances, healthcare, and legal affairs.

Getting emancipated is not easy, and courts don't grant it casually. Most states require the minor to be at least 14 to 16, to be living apart from their parents, and to prove they can support themselves financially through documented, legal income — government assistance doesn't count. The minor files a petition with a court, and a judge evaluates whether emancipation genuinely serves the minor's best interests, not just whether the minor wants to be free of parental rules.

Some situations trigger automatic emancipation without a court petition. Marriage and active-duty military service are the most common examples. In those cases, the minor's new legal status effectively supersedes parental control by operation of law, without needing a judge to formally approve the change.

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Source: legalclarity.org/at-what-age-can-a-child-make-their-own-decisions



TEXAS FAMILY CODE · TITLE 2 · CHAPTER 32

§ 32.003. Consent to Treatment by Child

Current as of January 1, 2024

(a) A child may consent to medical, dental, psychological, and surgical treatment for the child by a licensed physician or dentist if the child:

(1) is on active duty with the armed services of the United States of America;

(2) is:

(A) 16 years of age or older and resides separate and apart from the child's parents, managing conservator, or guardian, with or without the consent of the parents, managing conservator, or guardian and regardless of the duration of the residence; and

(B) managing the child's own financial affairs, regardless of the source of the income;

(3) consents to the diagnosis and treatment of an infectious, contagious, or communicable disease that is required by law or a rule to be reported by the licensed physician or dentist to a local health officer or the Texas Department of Health, including all diseases within the scope of Section 81.041, Health and Safety Code;

(4) is unmarried and pregnant and consents to hospital, medical, or surgical treatment, other than abortion, related to the pregnancy;

(5) consents to examination and treatment for drug or chemical addiction, drug or chemical dependency, or any other condition directly related to drug or chemical use;

(6) is unmarried, is the parent of a child, and has actual custody of his or her child and consents to medical, dental, psychological, or surgical treatment for the child; or

(7) is serving a term of confinement in a facility operated by or under contract with the Texas Department of Criminal Justice, unless the treatment would constitute a prohibited practice under Section 164.052(a)(19), Occupations Code.

- (b) Consent by a child to medical, dental, psychological, and surgical treatment under this section is not subject to disaffirmance because of minority.
- (c) Consent of the parents, managing conservator, or guardian of a child is not necessary in order to authorize hospital, medical, surgical, or dental care under this section.
- (d) A licensed physician, dentist, or psychologist may, with or without the consent of a child who is a patient, advise the parents, managing conservator, or guardian of the child of the treatment given to or needed by the child.
- (e) A physician, dentist, psychologist, hospital, or medical facility is not liable for the examination and treatment of a child under this section except for the provider's or the facility's own acts of negligence.
- (f) A physician, dentist, psychologist, hospital, or medical facility may rely on the written statement of the child containing the grounds on which the child has capacity to consent to the child's medical treatment.

Source: Texas Family Code § 32.003, retrieved via FindLaw (findlaw.com), updated by FindLaw Staff. Statutory cross-references: Section 81.041, Health and Safety Code; Section 164.052(a)(19), Occupations Code.

Xtra Magazine

YOUTH *Explainer — provides context or background, definition and detail on a specific topic.*

How 'mature minor' laws let trans kids make their own decisions

Canadian law lets some youth make medical or legal decisions for themselves, but how does it work?

By **Florence Ashley** • October 3, 2024



Credit: Getty Images; Elham Numan/Xtra

In 2023, Canada saw a number of anti-trans laws and policies adopted or proposed. Two provinces—Saskatchewan and New Brunswick—gave parents a veto over their child's choice of name and pronouns in schools under the age of 16. Alberta is planning to adopt a similar law and supplement it with a ban on gender-affirming care for youth under the age of 16, among other things.

In discussions on these laws and policies, you may have heard someone say that they violate the rights of "mature minors." The rights of mature minors were notably mentioned by Alberta law professors—including myself—in an open letter back in February. In the open letter, we claim that Alberta's announced ban on gender-affirming care for minors would violate the rights of mature minors to make medical decisions.

You might be asking yourself: what are mature minors, and how does being a mature minor impact the rights that youth have?

The quick-and-short version is that Canadian law lets minors make decisions for themselves—regardless of their parents' opinions—if they have enough intelligence, understanding and maturity to make free and informed decisions. Youth who satisfy these criteria are called mature minors.

The rights of mature minors primarily come up in the context of medical decisions, but they are also relevant to other issues—in theory, it applies to any personal decision in the child's life, including one's choice of name, pronouns, clothing, etc.

The emergence of mature minor doctrine and adoption into Canadian law

The concept of "mature minor" was adopted into Canadian law following a 1985 decision of the British House of Lords called *Gillick*. In *Gillick*, a conservative activist sued the local government for prescribing contraception to some youth under 16 without parental consent, as a matter of policy. The case went all the way to the highest court in England and Wales, which affirmed that doctors didn't need parental consent to prescribe contraception to minors who are sufficiently intelligent and mature.

According to the judges, saying that people remain under the complete control of their parents until 18 years old is "contrary to the ordinary experience of mankind," and "most wise parents relax their control gradually as the child develops." Accordingly, they held that minors could consent to medical treatment on their own, without parental consent, once they "achieve[d] sufficient understanding and intelligence to understand fully what is proposed."

This was a watershed moment for English law, but its influence didn't stop there. Soon after, Canadian courts began to cite the case and adopt its reasoning. It wasn't until 2009, however, that it was fully enshrined in Canadian law.

That year, the Supreme Court of Canada released its decision in a case called *AC v. Manitoba*. In that case, the government sought to force a 14-year-old to undergo blood transfusions even though she vehemently objected to blood transfusions for religious reasons. The law allowed medical care to be imposed if it was in the minor's best interests.

The Supreme Court answered that requiring parental consent for the medical decisions of a mature minor would violate their right to life, liberty and security of the person under section 7 of the Canadian Charter of Rights and Freedoms.

Although the law didn't explicitly talk about the mature minor doctrine developed in *Gillick*, the Supreme Court said that it was implicitly incorporated into the concept of the best interests of the minor. The more mature a person is, the more their decisions are inherently in their best interests. Once mature enough, there is no difference between a youth's perspective and best interests.

According to the judges: "If, after a careful and sophisticated analysis of the young person's ability to exercise mature, independent judgment, the court is persuaded that the necessary level of maturity exists, it seems to me necessarily to follow that the adolescent's views ought to be respected."

Holding otherwise would violate youths' constitutional right to liberty as well as Canada's obligations under the Convention on the Rights of the Child, which says that the older and more mature a minor is, the more weight their opinion should have.

This means that, for most purposes, mature minors must be treated the same as adults. The 14-year-old had won the right to try and demonstrate her own maturity and, in doing so, potentially refuse blood transfusions.

How to know if someone is a mature minor

In *AC v. Manitoba*, the Supreme Court ultimately declined to decide whether the 14-year-old could refuse blood transfusions. They kicked the can down the road, telling lower courts to figure out whether she was mature enough using the new legal framework they just developed.

Telling if someone is a mature minor is a matter of determining whether they have sufficient intelligence, understanding and maturity to provide genuinely informed consent—what courts sometimes call "true, stable and independent choices."

The Supreme Court gave a list of questions to help us ascertain maturity in the context of medical decisions: Can the minor understand what the treatment does and why it's being offered? Can they appreciate the risks and benefits of making different choices? Is the minor's opinion based on their own core values and beliefs? Are they being influenced by others? Do they have emotional vulnerabilities or psychiatric conditions that may impact their ability to make a mature and independent decision? Does the condition being treated negatively impact their ability to make a decision?

Importantly, the assessment has to be based on that specific minor, that specific decision, and at that specific point in time. An adolescent may be sufficiently mature for a specific decision in one context yet be considered immature regarding another decision or in another context.

Typically, greater understanding and sophistication is required for decisions involving more severe or far-away consequences—such as consequences that wouldn't be felt for years or decades. For example, courts and doctors more readily let minors consent to an HPV vaccine than to making an emergency life-or-death medical decision.

In the context of trans youth, this means that choosing your name and pronouns doesn't require as much understanding as deciding to take hormone therapy, since it changes your body. This doesn't mean that minors can't consent to hormone therapy—they often can. It just means that they need more sophisticated thinking than for names and pronouns.

While the matter is solely one of maturity in theory, the law indirectly allows agreement and disagreement with the minor's decision to influence whether they are considered mature.

The Supreme Court is of the view that minors don't need as much understanding to consent to treatment than to refuse, since consent aligns with the advice of doctors with "years of study and experience" whereas refusal ignores that advice.

Maturity will be more carefully evaluated for serious decisions than smaller ones. This makes it easier for adults to impose their views on minors for more significant choices.

Assessments of maturity in practice

Assessments of maturity are typically done by doctors who have little legal knowledge and may not even have heard of *AC v. Manitoba*. Instead, they tend to rely on approaches taught in medical school and standardized questionnaires.

Doctors traditionally divide maturity assessments into four components: communication, understanding, appreciation and reasoning. First, doctors ask the minor to indicate their choice to ascertain whether they can communicate their decisions. Second, they will ask the minor to explain back information about the medical condition and treatment. Third, they will ask the minor to describe their views regarding the medical condition, proposed treatment and likely outcomes. And fourth, they will ask the minor to compare treatment options and consequences, and offer reasons for their decision.

Oftentimes, the answer will be straightforward. The minor will either be evidently mature or immature. There is, however, room for much grey area that neither law nor medicine has managed to clarify.

Figuring out the precise line between mature and immature is more of an art than a science. It is a fuzzy line—and, arguably, a somewhat arbitrary one. However, some studies suggest that youth are normally able to consent to medical research by 10.4 years old, which is younger than many people assume. We also know from research that trans youth seeking hormone therapy usually have the level of maturity and understanding expected of mature minors.

Who decides?

When courts apply the mature minor doctrine, they typically do so by looking at the assessment conducted by a medical doctor. Most often, the person is either the child's doctor or a child psychiatrist who became involved at the request of the child's doctor.

However, going to court is rarely necessary. Courts have explained that doctors are usually allowed to act on the consent of minors they consider mature. In some provinces, this situation is made explicit in the law. In other provinces, it is considered implicit.

More generally, applying mature minor doctrine is done by whoever is responsible for making decisions in the best interests of the child. This includes teachers, since they are temporarily granted decisional authority by parents and under education law.

Because the law says that parents must exercise their parental authority "in a manner that is consistent with the best interests of the child," they are supposed to respect the decisions of their mature children. This notably means that they—and teachers—are supposed to respect trans youths'

names and pronouns if they are mature minors, regardless of what they think about it. If they don't, children and trusted adults can go to court to contest the parent's decision. Unfortunately, they rarely do so due to how expensive and burdensome the legal process can be.

While mature minors have a clear constitutional right to make decisions for themselves, it is far from clear how burdensome the maturity assessment process must be before this right is violated. Could governments pass a law saying that no minor will be considered mature unless they go to court and are supported by three psychiatrists? Probably not, but nobody knows for sure.

Confidentiality

Another point of uncertainty is whether mature minors can demand confidentiality with regard to their parents. Can mature minors ask doctors and teachers to keep information from their parents?

In Canada, medical practice typically assumes that mature minors have a right to confidentiality and some provinces have adopted laws that say so. According to the healthcare tribunal of British Columbia, a mature minor "has the same right to privacy as an adult patient." This assumption is very important for minors seeking birth control and abortion, who often don't want their parents to know. However, the question has not been comprehensively explored by courts.

There are good reasons to believe that the Supreme Court would indeed recognize mature minors' right to confidentiality. If it is in minors' best interests to respect their decisions, as *AC v. Manitoba* teaches us, shouldn't that also be the case for the decision to keep certain matters private?

This is how English courts have resolved the question. In the 2006 case called Axon, the mother of two teenage daughters sued the government for telling doctors that minors have confidentiality rights regarding contraception, abortion and sexual health. The High Court said that the government guidance was right: mature minors have a right to medical privacy and parents do not have a right to know whether their child has accessed reproductive healthcare. Hopefully Canadian courts would say the same.

It is also uncertain whether the same test will apply to the right to make decisions and the right to confidentiality. Since parents often help minors make better decisions, courts might decide to develop a more stringent test for confidentiality.

Trans mature minors

The mature minor doctrine applies to trans youth. Trans mature minors can make decisions regarding gender-affirming care.

They can also make decisions regarding their gender identity and expression, such as choosing their pronouns, name, clothing, etc. However, these decisions are more difficult to enforce than medical ones since there isn't a clear, explicit and effective legal structure in place to govern non-medical decisions.

As writer Ned Lecic and retired judge Marvin A. Zuker put it, "children are physically, emotionally and, for the most part, financially dependent on their parents, and there is no public authority that monitors the day-to-day actions of parents. The practical result is that, day to day, parents may effectively enforce any rules they see fit in their own homes."

If a parent or teacher refuses to respect a trans youth's pronouns, name or clothing choices, minors can, in principle, ask courts to intervene. But given how expensive and convoluted courts are, this is rarely realistic.

Laws that interfere with trans youth's decisional autonomy can also be challenged under the Canadian Charter. Recent laws and policies targeting trans youth in education have been promptly challenged in court, and mature minor doctrine is one of the arguments brought up in those lawsuits.

Unfortunately, the mature minor doctrine has the same vulnerability as most rights of the Canadian Charter. Like almost all Charter rights, it can be overridden by governments using the Notwithstanding Clause.

In provinces where the notwithstanding clause isn't invoked, however, the constitutional rights of trans mature minors remain strong.

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Medical Rights for Teens and Kids: When Parents Decide and When They Don't

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In the United States legal system, parents are entrusted with making medical decisions for their children.

This authority, however, is conditional. It is shaped by federal and state laws, landmark Supreme Court rulings, and evolving ethical standards.

A teenager's right to consent to their own healthcare can change dramatically depending on where they live, their personal circumstances, and the specific medical service they need.

The General Rule: Why Parents Decide

The bedrock of medical law for children in the United States is the principle of parental consent. This long-standing rule is rooted in legal tradition, ethical considerations, and a societal understanding of the family unit.

Before examining the many exceptions, it's essential to understand why this principle exists and how it works.

Defining a Minor

The legal system creates a bright line between childhood and adulthood known as the "age of majority." Under federal healthcare regulations and the laws of most states, a "minor" is legally defined as anyone under the age of 18.

Once an individual reaches this age, they are considered an adult for most legal purposes, including the right to make their own healthcare decisions.

There are, however, a few state-level exceptions to this standard. The age of majority is 19 in Alabama and Nebraska, and 21 in Mississippi, although Mississippi law specifically sets 18 as the age of consent for healthcare services.

For anyone legally defined as a minor, the general rule is that a healthcare provider cannot administer medical treatment without first obtaining legally valid informed consent. This consent must come from a person with the legal authority to make decisions on the minor's behalf, which typically includes:

A biological parent. A legal guardian who can provide official court documentation of their status. An individual holding a valid medical power of attorney (MPOA) for the minor.

This requirement is a foundational element of U.S. healthcare law, designed to protect children who are legally presumed to lack the capacity to make complex medical decisions for themselves.

The Foundations of Parental Authority

The authority granted to parents is not arbitrary; it is built on deep-seated legal and ethical principles. Legally, the right of parents to direct the care and upbringing of their children is considered a fundamental constitutional right, stemming from the right to privacy in family matters and long-standing common law traditions.

This authority is broad, even including the right to refuse or discontinue treatments, including those that may be life-sustaining.

Ethically, this power is vested in parents based on the presumption that they are the individuals who care most about their child and are uniquely positioned to understand the child's needs, values, and personal history. This intimate knowledge allows them to make decisions that are in the child's "best interests."

Furthermore, because a child's medical journey often affects the entire family, parents can integrate broader family values and considerations into the decision-making process. The American Medical Association (AMA) frames this as a dual responsibility for parents: to safeguard their children's physical health and well-being and to nurture their developing personhood and capacity for autonomy.

This "best interests" standard, however, is not an unlimited grant of power; it is also the primary justification for limiting parental authority. The power delegated to parents is conditional.

The state, under a legal doctrine known as *parens patriae* (Latin for "parent of the nation"), retains the ultimate authority to protect the welfare of children. When parental decisions are deemed "potentially dangerous," "imprudent, neglectful, or abusive," or place a child at "significant risk of serious harm," medical providers and the state have an ethical and legal duty to intervene.

A landmark 1944 Supreme Court case, *Prince v. Massachusetts*, affirmed that the state could override parental decisions, even those based on religious beliefs, when the refusal of medical treatment would likely result in the child's death or significant disability.

This creates a foundational tension in the law: the very principle that grants parents their authority—acting in the child's best interest—is the same principle that allows the state to take that authority away. This inherent conflict is the source of nearly every exception to the parental consent rule.

From Battery to Self-Determination

The modern concept of medical consent did not originate with children's rights but with the rights of adults, rooted in the legal concept of battery—an unwanted physical touching. In the early 20th century, a series of pivotal court cases began to codify the right of patients to control what happens to their own bodies.

In *Mohr v. Williams* (1905), a patient consented to surgery on her right ear, but the surgeon, upon finding the left ear to be in worse condition, operated on it instead without her consent. The court ruled against the surgeon, establishing that a doctor cannot assume consent.

Similarly, in *Pratt v. Davis* (1906), a court found a surgeon liable for performing a hysterectomy on a patient who had not consented to that specific procedure.

These cases culminated in the famous 1914 opinion by Judge Benjamin Cardozo in *Schloendorff v. Society of New York Hospital*. A woman who had consented only to an examination was given a hysterectomy against her express wishes.

In his ruling, Cardozo wrote the defining words of patient autonomy: “Every human being of adult years and sound mind has a right to determine what shall be done with his own body; and a surgeon who performs an operation without his patient’s consent commits an assault, for which he is liable in damages.”

This history is critical for understanding minors’ rights. It establishes that the default rule for adults is self-determination. Therefore, the default rule for minors—parental consent—is fundamentally an exception to this principle of autonomy.

The entire legal framework governing medical rights for teens and kids is a continuous negotiation over when, and under what circumstances, a young person transitions from being under the protection of the parental consent rule to having the full autonomy granted to adults.

When Minors Gain Legal Adulthood

While the age of majority marks the standard transition to full medical autonomy, the law recognizes that some minors, due to their life circumstances or demonstrated maturity, should be granted the rights of an adult sooner.

There are two primary legal pathways for this to occur: emancipation and the mature minor doctrine. These mechanisms allow a minor to “graduate” from the parental consent rule and become their own healthcare advocate.

The Emancipated Minor

Emancipation is a formal legal process through which a minor is declared an adult in the eyes of the law, freeing them from the control of their parents or guardians. In turn, parents are released from their legal responsibilities to the child, including financial support.

An emancipated minor has the same capacity as an adult to make medical decisions, including the right to consent to or refuse treatment without any parental involvement or notification.

The specific laws and procedures for emancipation vary significantly from state to state, creating a patchwork of different requirements across the country. However, there are several common pathways by which a minor can achieve this status:

Marriage: In most states, a minor who is legally married is automatically considered emancipated for the purpose of medical consent.

Military Service: A minor on active duty in the U.S. armed forces is also generally considered emancipated.

Court Order: A minor can petition a court for a formal declaration of emancipation. To be successful, the minor typically must prove to the court that they are living separately from their parents and are capable of managing their own financial affairs.

Being a Parent: In many jurisdictions, a minor who is a parent can consent to medical care for their own child, and in some cases, this status also grants them the right to consent to their own medical care.

Once emancipated, a minor is not only empowered to make their own medical decisions but is also legally responsible for paying for their own medical care. Under the Health Insurance Portability and Accountability Act (HIPAA), an emancipated minor is treated as the “individual” and holds the right to control access to their own protected health information.

The Mature Minor Doctrine

A second, more nuanced pathway to self-consent is the “mature minor” doctrine. This is a legal rule, recognized either by state statute or by court decisions, that allows a minor who is not emancipated to consent to specific medical treatments if they can demonstrate the capacity to do so.

Unlike emancipation, which confers broad adult rights, the mature minor doctrine is typically applied on a case-by-case basis. A physician or a court might determine that a 16-year-old is mature enough to consent to a low-risk procedure but not to a complex, high-risk surgery.

This doctrine represents a philosophical shift in the law. The general rule of parental consent is status-based—a person’s rights are determined by their legal status as a “minor” or an “adult.” Emancipation is also status-based, as it changes the minor’s legal category.

The mature minor doctrine, in contrast, is capacity-based. It argues that the right to make a decision should be tied to an individual’s actual ability to understand the situation and its consequences, regardless of their age or legal status.

This approach aligns more closely with modern bioethical principles that prioritize patient autonomy and informed consent.

The criteria used to determine if a minor is “mature” vary by state but generally focus on assessing the minor’s ability to give a truly informed consent. Factors considered often include:

Age: While there is no universal age, the doctrine is most commonly applied to older adolescents, typically those 14 or 16 years of age and older.

Understanding: The minor’s ability to comprehend the nature of their medical condition, the proposed treatment, the risks and benefits, and any available alternatives.

Experience and Education: The minor’s life experience, education level, and general conduct.

Judgment: The minor’s ability to weigh the options and make a reasoned decision.

Despite its ethical appeal, the application of the mature minor doctrine is highly inconsistent across the United States, revealing a deep societal and legal ambivalence about adolescent autonomy.

A handful of states, such as Arkansas and Nevada, have formally enacted the doctrine into their state laws. Several others, including Pennsylvania, Illinois, and Massachusetts, recognize it as a part of their common law through judicial precedent.

However, many states have no such doctrine, and some have even taken steps to limit its reach. For example, Tennessee, which had a long-standing mature minor doctrine based on a “Rule of Sevens” (presuming capacity for those 14 and older), recently passed a law that explicitly counteracts it for immunizations, now requiring parental consent in all cases.

This legislative rollback, driven by political debates over “parental rights,” illustrates the fragile and contested nature of the doctrine. It exists in a state of tension—a progressive, capacity-based view of autonomy that has not been fully embraced by a legal system that remains hesitant to cede parental authority.

Exceptions for Sensitive Services

Beyond the pathways that grant minors broad adult status, the most common way a minor can consent to their own care is through specific, service-based exceptions carved out by state laws.

These laws create protected zones where adolescents can seek confidential treatment for sensitive health issues without parental involvement.

Why Create Exceptions?

The primary motivation behind these exceptions is a pragmatic public health concern. Lawmakers and public health experts have recognized that for certain sensitive issues—such as sexual health, mental health, and substance abuse—requiring parental consent can be a significant barrier to care.

Adolescents, fearing judgment, punishment, or family conflict, may avoid seeking necessary medical help altogether if they are forced to disclose their situation to a parent.

This expansion of minors’ medical authority gained significant momentum in the 1970s, spurred by a series of U.S. Supreme Court rulings that extended the constitutional right to privacy to cover a minor’s decisions regarding contraception and abortion.

These laws, therefore, represent a calculated balance. They acknowledge the general importance of parental involvement but conclude that in these specific, high-stakes contexts, the societal benefit of preventing teen pregnancies, treating STDs, and addressing mental health crises outweighs the default principle of parental authority.

Sexual and Reproductive Health

Nowhere are the exceptions to parental consent more prevalent than in the area of sexual and reproductive health.

Sexually Transmitted Diseases (STDs) and HIV: This is the most universally accepted exception. All 50 states and the District of Columbia have laws that explicitly permit minors to consent to the diagnosis and treatment of STDs, including HIV.

While the specifics can vary—for example, California allows minors aged 12 and older to consent for this care—the core principle of confidential access is recognized nationwide.

Contraception and Family Planning: Many states have laws or policies that explicitly authorize minors to consent to contraceptive services. This access is strongly reinforced by federal policy.

The Title X Family Planning Program, a major source of federal funding for family planning clinics, has been required to provide services to all individuals who need them, regardless of age, since its inception in 1970.

However, this area is not without controversy. Some states, notably Texas and Utah, have passed laws that prohibit the use of state funds to provide prescription contraceptives to minors without parental consent, creating a complex funding-dependent system of access.

Pregnancy and Prenatal Care: A minor who is pregnant is generally empowered to make her own medical decisions related to that pregnancy. This includes consenting to prenatal care, labor and delivery services, and postpartum care without needing parental permission.

Minor as a Parent: Once a minor becomes a parent, their legal status regarding medical consent shifts dramatically. In most jurisdictions, a minor parent can legally consent to medical treatment for themselves and for their child.

This can lead to a paradoxical legal situation. For instance, a 16-year-old mother could legally consent to a major surgical procedure for her infant, but she might not be able to consent to the very same procedure for herself if it were unrelated to her pregnancy, unless another exception applied.

Mental Health and Substance Abuse Treatment

Recognizing the critical importance of early intervention, a majority of states have enacted laws that allow minors to consent to their own outpatient mental and behavioral health services, as well as treatment for substance abuse.

However, the laws in this area are particularly varied and often contain specific limitations. The age at which a minor can consent differs widely, with states setting the threshold at 12, 13, 14, or 16.

Furthermore, a minor's right to consent may not be absolute even within these exceptions. For example, in Maryland, a minor aged 12 or older can consent to mental health counseling and therapy. However, if that minor is under the age of 16, parental consent is still required for a doctor to prescribe psychiatric medications.

This demonstrates the granular nature of these laws, where the right to consent can depend on the minor's age and the specific type of treatment being offered (e.g., talk therapy versus prescription drugs).

Emergency Care and Special Circumstances

Medical Emergencies: The most clear-cut exception to parental consent is in the case of a medical emergency. This rule is universal across all states.

A healthcare provider is permitted—and expected—to provide necessary medical treatment to a minor without parental consent when a delay to obtain that consent would endanger the minor's life or seriously threaten their health.

This common-law principle is bolstered by the federal Emergency Medical Treatment and Active Labor Act (EMTALA), which requires any hospital receiving Medicare funding to provide a medical screening exam and stabilizing treatment to any person presenting with an emergency medical condition, regardless of their age or ability to pay.

Homeless and Runaway Youth: A growing number of states are recognizing the unique vulnerability of homeless and runaway youth, who often lack access to a parent or guardian to provide consent.

To address this gap, states like New York and Washington have passed specific laws that empower these minors to consent to their own general medical, dental, and hospital care.

For example, a 2023 law in New York explicitly allows homeless youth to consent to a wide range of health services, ensuring they are not denied care due to their circumstances.

How the Supreme Court Shaped Minor’s Rights

The legal landscape of minors’ medical rights has been profoundly shaped by the U.S. Supreme Court. The Court’s rulings have navigated the turbulent intersection of parental authority, a minor’s developing autonomy, and the state’s duty to protect children.

These landmark cases did not create a single, simple rule but instead established a complex and context-dependent framework that defines the constitutional boundaries of the issue.

A Constitutional Balancing Act

The legal questions surrounding a minor’s right to consent are not simple because they involve a clash of powerful, constitutionally recognized interests. The Supreme Court’s jurisprudence in this area can be understood as a continuous effort to balance three competing claims:

The Parent’s Right: The Court has long recognized that parents have a fundamental liberty interest, protected by the Due Process Clause of the Fourteenth Amendment, in the care, custody, and control of their children. This includes the right to guide their upbringing and make important decisions on their behalf.

The Minor’s Right: The Court has also affirmed that minors are “persons” under the Constitution and possess their own constitutional rights. These include a right to privacy and bodily integrity, which, while not as extensive as an adult’s, can limit the power of both parents and the state.

The State’s Interest: The government has a legitimate and compelling interest in the well-being of its citizens, particularly children. Under its *parens patriae* authority, the state can intervene to protect a child’s health and safety, especially when parental decisions might lead to harm.

The outcomes of the landmark cases below are a direct result of how the Court weighed these three interests in different medical contexts.

Key Supreme Court Precedents

The most significant legal battles over minors’ consent have been fought in the highly contentious arena of reproductive rights, leading to a series of foundational precedents.

Planned Parenthood of Central Missouri v. Danforth (1976)

Background: This case challenged a Missouri law enacted after *Roe v. Wade* that imposed several restrictions on abortion, including a requirement that an unmarried minor obtain the written consent of one of her parents before the procedure.

Ruling: The Supreme Court struck down the parental consent provision. The majority opinion, written by Justice Blackmun, reasoned that since the state itself could not prohibit an abortion during the first trimester, it could not delegate that power to a third party, including a parent.

The Court declared that the state does not have the constitutional authority to give a parent “an absolute and possibly arbitrary veto” over the decision of the physician and their minor patient.

Significance: Danforth was a groundbreaking decision. It was the first time the Supreme Court explicitly ruled that a minor’s constitutional right to privacy could, in the context of abortion, supersede a parent’s desire to control that decision. It firmly established that parental authority is not absolute.

Bellotti v. Baird (1979)

Background: This case involved a Massachusetts statute that was more complex than the one in Danforth. It required an unmarried minor to obtain consent from both parents for an abortion.

If one or both parents refused, the minor could seek approval from a superior court judge. However, the law was flawed: the judge could still deny the abortion even if they found the minor to be mature and competent, and the law required that parents be notified of the court proceeding in every instance.

Ruling: The Court found the Massachusetts law unconstitutional. It identified two key defects. First, it allowed a judge to overrule the decision of a minor who was demonstrably mature and capable of making the choice for herself. Second, it provided no pathway for a minor to go to court confidentially, without her parents’ knowledge.

Significance: Bellotti is arguably the most important case for defining the modern legal framework for parental involvement laws. In its decision, the Court outlined the constitutional requirements for any such law.

It held that if a state wants to require parental consent, it must provide an alternative “judicial bypass” procedure. This procedure must allow the minor to demonstrate to a judge, in a confidential and expedited manner, that either: 1) she is mature and well-informed enough to make the abortion decision on her own, OR 2) even if she is not, the abortion is in her “best interests.”

This framework remains the constitutional standard that nearly all current state parental involvement laws are built upon.

Parham v. J.R. (1979)

Background: Decided in the same year as Bellotti, this case dealt with a very different issue: the “voluntary” commitment of a minor to a state mental institution by their parents or guardians.

A class-action lawsuit filed on behalf of minors argued that this process, which did not require a formal hearing, violated their due process rights.

Ruling: In a stark departure from the reasoning in the abortion cases, the Court upheld Georgia’s procedures. It heavily emphasized the legal presumption that parents act in their children’s best interests and found that requiring a formal, adversarial hearing would create an “unacceptable intrusion into the parent-child relationship.”

However, the Court did not grant parents absolute power. It ruled that a child’s due process rights require some protection, which could be satisfied by having a “neutral factfinder”—such as a staff physician at the institution—review the admission to determine if the child’s commitment is medically necessary and appropriate.

Significance: Parham strongly affirmed parental authority in the context of mental healthcare. When viewed alongside Danforth and Bellotti, it reveals that the Supreme Court has not created a single, unified theory of minors’ rights.

Instead, it has established a bifurcated system. In the realm of reproductive health, where a minor’s own fundamental privacy right is paramount, the Court is highly skeptical of parental authority and has erected procedural safeguards like the judicial bypass to limit it.

In other areas, such as mental health commitment, the Court is far more deferential to parental authority, viewing the decision as parents seeking necessary care for their child, and requires only minimal procedural checks.

This creates a legal paradox: a minor may have more constitutionally protected autonomy to decide to have an abortion against her parents’ wishes than she does to refuse commitment to a mental institution that her parents desire.

This demonstrates that “minors’ rights” are not a monolithic concept but a collection of context-dependent rules driven by which competing interest—the parent’s, the minor’s, or the state’s—the Court prioritizes in a given situation.

In the Doctor’s Office: Ethics and Practical Challenges

While laws and court cases define the legal boundaries, the real-world application of minors’ medical rights happens in the examination room. Here, physicians, patients, and parents navigate a complex terrain of ethical duties, privacy concerns, and practical hurdles that can make confidential care a significant challenge.

The Doctor’s Ethical Compass

Beyond legal requirements, physicians are guided by the ethical principles of their profession. Major medical organizations, including the American Medical Association (AMA) and the American Academy of Pediatrics (AAP), have established guidelines that often encourage greater respect for adolescent autonomy than the law may strictly mandate.

Both the AMA and the AAP emphasize a physician’s ethical duty to promote the “developing autonomy” of their minor patients. This is not about simply waiting for a patient to turn 18. Instead, it is an active process of involving children and adolescents in discussions about their health to a degree that is “commensurate with their abilities.”

Physicians are ethically encouraged to talk with minor patients about why they may be reluctant to involve their parents, to correct any misconceptions, and to offer to facilitate conversations between the teen and their family whenever it is safe and appropriate to do so.

When a minor who is judged to be competent requests confidential care for a sensitive service—and the law does not explicitly require parental involvement—the AMA’s Code of Medical Ethics is clear: the physician should ordinarily respect the minor’s wishes and provide the confidential care.

If a particular physician is uncomfortable doing so, their ethical obligation is not to deny care but to refer the minor to another provider or clinic where confidential services can be obtained.

Assent vs. Consent

A crucial distinction in pediatric ethics is the difference between legal consent and ethical assent.

Consent is the legally valid authorization for medical treatment. For a minor, this authority is typically held by a parent or guardian.

Assent is the affirmative, willing agreement of a minor to participate in a medical procedure. While it is not legally binding in most cases, it is a critical ethical requirement.

Seeking a child's assent is a way of showing respect for them as a developing person, not just an object of medical treatment. It is a vital part of building a trusting and positive physician-patient relationship.

As a child matures, the ethical weight of their assent—or dissent—grows. The AAP advises that the refusal of assent from a competent older child or adolescent should be taken very seriously and explored, as it may indicate a misunderstanding or a deep-seated fear that needs to be addressed.

The practice of seeking assent serves as the clinical bridge between a young child who has no say in their care and an adult who has full legal authority, operationalizing the ethical duty to foster autonomy.

The Privacy Paradox

The promise of confidential care for minors faces a significant real-world obstacle: the structure of the U.S. health insurance system.

The Health Insurance Portability and Accountability Act (HIPAA) Privacy Rule is the federal law that governs the confidentiality of protected health information. As a general rule, HIPAA allows a parent to access their child's medical records because they are considered the child's "personal representative."

However, HIPAA includes a critical provision that defers to state law. If a state law permits a minor to consent to a particular type of care on their own (for example, STD treatment), then for that specific service, the parent is not considered the personal representative. In that situation, the minor alone controls who can access their medical information.

While this legal structure seems to protect confidentiality, it often breaks down in practice. The problem arises when the family's health insurance is used to pay for the confidential service.

The insurance company, in its normal course of business, sends an Explanation of Benefits (EOB) statement to the primary policyholder—who is almost always the parent. This EOB document typically lists the patient's name, the date of service, the provider, and the type of service rendered.

This routine piece of mail can inadvertently and completely breach the minor's confidentiality, informing parents about care the teen sought to keep private.

This creates a "confidentiality gap." The law may grant a legal right to confidential care, but the practical realities of the payment system can render that right meaningless. The fear of an EOB being sent home is a well-known deterrent that can discourage teens from seeking the very care these laws were designed to encourage.

This is not strictly a legal failure but a systemic one, where the logic of the insurance system (informing the policyholder) conflicts directly with the logic of public health law (ensuring confidentiality for sensitive services).

While workarounds exist—such as paying out-of-pocket, seeking care at federally funded Title X clinics that have systems to ensure billing privacy, or enrolling in state-specific benefit programs designed for confidential care—they are not universally available or known.

This gap highlights a crucial point: a legal right is only as strong as the system that supports it, and for many American teens, the promise of medical privacy remains fragile.

A Patchwork of Laws: Medical Consent Across the 50 States

While federal laws and Supreme Court decisions provide a national framework, the day-to-day rules governing a minor’s right to consent to medical care are overwhelmingly determined at the state level.

This has resulted in a complex and often contradictory “patchwork” of laws, where the rights of a teenager can change dramatically by simply crossing a state line.

Why State Law is Key

There is no single “U.S. law” on minor medical consent. The federal government sets a baseline with laws like HIPAA and EMTALA, and the Supreme Court establishes broad constitutional boundaries, but the specific details—such as the age at which a minor can consent for mental health care, whether a “mature minor” doctrine is recognized, or what hoops a teen must jump through for an abortion—are decided by each state’s legislature and courts.

This means that the most critical piece of information for any parent, teen, educator, or healthcare provider is an understanding of the specific laws in their own state.

The following table provides a high-level, comparative snapshot of these variations across all 50 states and the District of Columbia. It serves as a practical starting point for understanding local rules and demonstrates the immense diversity in how states approach this issue.

State-by-State Snapshot of Minor Consent Laws

This table summarizes key provisions related to a minor’s ability to consent to healthcare. The information is primarily drawn from comprehensive legal reviews by the National Center for Youth Law and the Guttmacher Institute.

It is intended as a guide and is not a substitute for legal advice. Laws are subject to change, and users should verify information with primary state sources.

State	Age of Majority	General Medical Care (Minor's Consent)	Contraceptive Services	STD/HIV Diagnosis & Treatment	Outpatient Mental Health	Abortion (Parental Involvement)
Alabama	19	Age 14+ or HS grad	Not specified	Any age (VD); Age 12+ (STD)	Age 14+	Consent Required
Alaska	18	If parent unavailable/unwilling; living apart	Any age	Any age	Not specified	Not Required
Arizona	18	If homeless or married	Permitted by AG opinion	Any age	Age 12+ (for sexual assault)	Consent Required
Arkansas	18	Mature Minor Doctrine	Any age	Any age	Not specified	Consent Required
California	18	Age 15+ if living apart; Age 12+ for vaccination	Any age	Age 12+	Age 12+	Not Required
Colorado	18	Age 15+ if living apart; Married	Any age	Any age	Age 12+	Notification Required
Connecticut	18	Not specified	Not specified	Any age	If certain conditions are met	Not Required
Delaware	18	Mature Minor recognized	Age 12+ (if pregnant/ exposed)	Age 12+	Not specified	Notification Required
Dist. of Columbia	18	Not specified	Any age	Any age	Any age	Not Required
Florida	18	If married or a parent	Any age	Any age	Age 13+	Consent Required
Georgia	18	If married or a parent	Age 12+	Age 12+	Age 17+	Notification Required
Hawaii	18	Age 14+ if not supported by parents	Age 14+	Age 14+	Age 14+	Not Required
Idaho	18	Mature Minor recognized	Age 14+	Age 14+	Age 14+ (for admission)	Consent Required
Illinois	18	Mature Minor Doctrine	Age 12+	Age 12+	Age 12+	Notification Required
Indiana	18	If married or emancipated	Any age	Any age	Age 16+	Consent Required
Iowa	18	If married or emancipated	Any age	Any age	Age 14+	Notification Required
Kansas	18	Mature Minor Doctrine; Age 16+ if parent unavailable	Permitted by AG opinion	Any age	Mature Minor can consent	Consent Required
Kentucky	18	If married or a parent	Age 16+	Any age	Age 16+	Consent Required
Louisiana	18	Mature Minor Doctrine	Age 12+	Any age	Age 12+	Consent Required
Maine	18	Mature Minor Doctrine	Age 12+	Any age	Age 12+	Not Required
Maryland	18	If married or a parent	Any age	Any age	Age 12+	Notification Required
Massachusetts	18	Mature Minor Doctrine	Any age	Any age	Age 12+	Consent Required
Michigan	18	If married or emancipated	Age 12+	Any age	Age 14+	Consent Required
Minnesota	18	If living apart	Any age	Any age	Age 16+	Notification Required
Mississippi	21 (18 for HC)	If married or a parent	Any age	Any age	Age 12+	Consent Required
Missouri	18	If married or a parent	Age 12+	Any age	Age 12+	Consent Required
Montana	18	HS Grad; Age 16+ if living apart	Age 16+	Any age	Age 16+	Notification Required
Nebraska	19	If married or emancipated	Not specified	Any age	Age 18+	Consent Required
Nevada	18	Mature Minor Doctrine	Any age	Any age	Age 12+	Notification Required
New Hampshire	18	Mature Minor Doctrine	Age 12+	Any age	Age 12+	Notification Required
New Jersey	18	If married or pregnant	Age 12+	Any age	Age 16+	Not Required
New Mexico	18	Age 14+ if living apart	Age 14+	Any age	Age 14+	Not Required
New York	18	If married, a parent, or homeless	Any age	Any age	Age 12+	Not Required

State	Age of Majority	General Medical Care (Minor's Consent)	Contraceptive Services	STD/HIV Diagnosis & Treatment	Outpatient Mental Health	Abortion (Parental Involvement)
North Carolina	18	If emancipated	Age 12+	Any age	Age 12+	Consent Required
North Dakota	18	If married or emancipated	Age 12+	Any age	Age 12+	Consent Required
Ohio	18	If married or a parent	Age 12+	Any age	Age 12+	Consent Required
Oklahoma	18	If married, a parent, or HS grad	Age 12+	Any age	Age 12+	Consent Required
Oregon	18	Age 15+	Age 15+	Any age	Age 14+	Not Required
Pennsylvania	18	Mature Minor Doctrine	Age 12+	Any age	Age 14+	Consent Required
Rhode Island	18	Age 16+	Not specified	Any age	Age 16+	Consent Required
South Carolina	18	Age 16+	Age 12+	Any age	Age 16+	Consent Required
South Dakota	18	If married or emancipated	Age 12+	Any age	Age 12+	Notification Required
Tennessee	18	Mature Minor Doctrine (limited for vaccines)	Age 12+	Any age	Age 16+	Consent Required
Texas	18	Age 16+ if living apart; Married; Parent	Age 16+	Age 13+	Age 16+	Consent Required
Utah	18	If married or a parent	Age 16+	Any age	Age 16+	Consent Required
Vermont	18	Age 12+ for some care	Age 12+	Age 12+	Age 12+	Not Required
Virginia	18	If married or emancipated	Age 12+	Any age	Age 14+	Consent Required
Washington	18	Mature Minor Doctrine	Age 12+	Age 14+	Age 13+	Not Required
West Virginia	18	Mature Minor Doctrine	Age 12+	Any age	Age 14+	Notification Required
Wisconsin	18	If married or a parent	Age 12+	Any age	Age 14+	Consent Required
Wyoming	18	If mature/emancipated	Not specified	Any age	Not specified	Not Required

This table is intended as a guide and is not a substitute for legal advice. Laws are subject to change, and users should verify information with primary state sources.